SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Cisplatin + fluorouracil every 4 weeks

Treatment of locally advanced or metastatic head and neck cancer. · NCCP 00314 · version 4b · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Cisplatin and fluorouracil are used to treat head and neck cancer. The treatment aim depends on the extent of the cancer and your wider plan; ask your team to record whether the aim is disease control, symptom relief or part of another treatment strategy.

Schedule
Both medicines are given on Days 1–4 of each 28-day cycle. Treatment continues while appropriate, until the cancer progresses or side effects become unacceptable; this protocol does not set a fixed number of cycles.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Nausea and vomiting
- ☐ Kidney injury and mineral changes
- ☐ Hearing loss or tinnitus, which can persist
- ☐ Nerve damage, which can persist
- ☐ Infusion reaction and effects on fertility

Fluorouracil
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Mouth and throat soreness
- ☐ Diarrhoea and dehydration
- ☐ Painful palms or soles
- ☐ Heart problems including chest pain
- ☐ Rare severe neurological or early excessive toxicity

Discuss the individual aim, expected benefit, alternatives and uncertainties; common, serious and lasting effects; infusion reactions and infection; fertility and pregnancy; treatment burden; support medicines and urgent contacts.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00314 v4b.

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